جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : كريم محمد على محمد
المحافظة : المنوفيه **الحي / المدينة :** شبين الكوم **العنوان :** 000000000
رقم البطاقة : 30506021702078 **رقم التليفون :** 01027485750 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 02-06-2005 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : اورام ليمفاويه
نتائج الأبحاث : (مرفق)
التشخيص النهائي : اورام ليمفاويه
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: كريم محمد علي محمد
National ID: 30506021702078

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of lymphoma for Contrast-enhanced MRI of the brain and orbits

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



كريم
محمد على محمد شحاته
الكو الاخضر
مركز شبين الكوم - المنوفية
٣٠٥٠٦٠٢١٧٠٢٠٧٨
٢٠٠٥/٠٦
LN7712832



٣٠٥٠٦٠٢١٧٠٢٠٧٨ ٢٠٢٦/٠١
حاصل على دبلوم صناعة
ذكر مسلم اعزب
١٢ البطاقة سارية حتى ٢٠٣٣/٠١/١١

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

المعهد القومي للأورام
جامعة القاهرة

Patient Name:	كريم محمد على محمد		
MRN:	10110225003669	Location:	عيادة العلاج الاشعاعي
D.O.B:	02/06/2005	Sex:	Male
IC No.:	30506021702078		

Surgical Pathology-Diagnosis

Slide/Block revision Pathology

Specimen Id	2502241727	**Collection Date**	24/02/2025 06:11 PM
Accession No	P-25-2384	**Received Date**	24/02/2025 06:12 PM
Sample Type	TISSUE	**Doctor**	العلاج الاشعاعي

CLINICAL HISTORY

Block review of left parieto occipital SOL excisional biopsy

GROSS DESCRIPTION

One referred paraffin block labelled by patients name only.

MICROSCOPIC DESCRIPTION

There is diffuse effacement by infiltrate formed of noncohesive large atypical round cells having medium to large nuclei and multiple nucleoli with abundant cytoplasm. Anaplasia is moderate & Mitosis is active. Marker study was done with tumor cells are positive for LCA, CD20, BCL6 while negative for CMYC, BCL2, SALL4, GFAP, SYNAPTOPHYSIN, NKX2, BCOR. INI-1 IS RETAINED. KI67 is positive in 20 % of tumor cells. CD3 stains reactive background T cells. p53 stained scattered few cells

DIAGNOSIS

Block review of left parieto occipital SOL excisional biopsy: Diffuse large B cell Non Hodgkin lymphoma

Category

Malignant primary site (single tumor)

Assistant Pathologist:
Dr. Riham Gabr

Registrar:

Signed by:
Dr. Hazem Abdelazim, MD (Lecturer)

Reported and electronically endorsed by Riham Gabr

[[DOCTYPE:BONE_SCAN]]
Bone Scan

MOHAMED ALI, KAREEM Patient ID: 10110225003669 DOB: 02-Jun-2005 Date: 01-Sep-2025
Patient Lastname: MOHAMED ALI
Patient Firstname: Kareem
HIS Patient ID: 10110225003669
RIS Patient ID: 367154
Date of Birth: 02/06/2005

Procedure: 93015, FDG PETCT
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
F18-FDG PET/CT STUDY
(01-09-2025)

CLINICAL HISTORY:
20-year-old male patient brain lesion; pathologically proven DLBCL, received CTH last was at 18-8-2025.
TECHNIQUE:
Approximately 1hour following intravenous injection of the radiopharmaceutical fluorodeoxyglucose (FDG), a total body PET-CT was acquired from base of skull to mid-thigh with the arms extended above the head. CT without contrast was acquired

Head and Neck:
* Status post-operative left parietal craniotomy.
* Left parieto-occipital brain hypometabolic cystic lesion measuring 5.2x6.2 cm for correlation with local MRI data.
* Right ventericulo-peritoneal shunt.
* The brain shows NO hypermetabolic focal lesions.
* Prominent Bilateral upper cervical LNs measuring about 1.2 cm with low grade FDG uptake of SUVmax~2.2, likely reactive.
* Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.

Chest:
* Left breast dense parenchyma with diffuse metabolic activity; likely gynecomastia.
* There is no hypermetabolic mediastinal lymphadenopathy.
* There is no hypermetabolic axillary lymphadenopathy.
* No FDG avid lung nodules. Few non FDG avid tiny right pulmonary nodules for FU.
* There are no active pleural or pericardial abnormalities.
* Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:
* There is no hypermetabolic abdominal or pelvic lymphadenopathy.
* No FDG avid hepatic focal lesions detected.
* The spleen and pancreas show normal FDG avidity.
* There are no active adrenal nodules.
* Physiologic FDG excretion is seen in the kidneys and bladder.

Musculoskeletal:
* Normal FDG activity is seen in the axial skeleton. No focal active blastic or lytic lesions noted.

Conclusion

[[DOCTYPE:LAB_TEST]]
Lab Test

THOR Laboratories Kft.

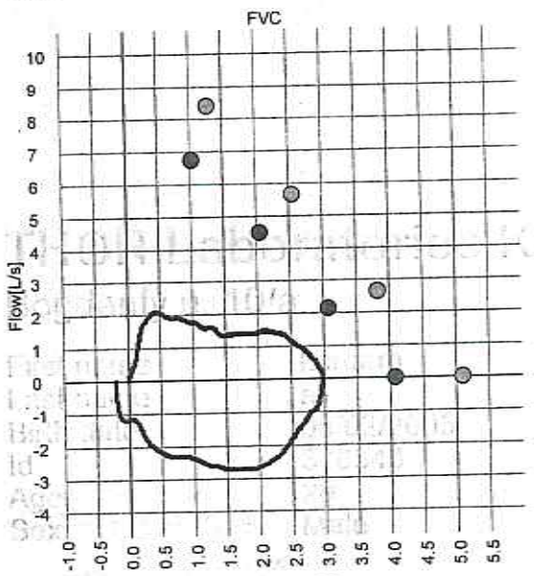
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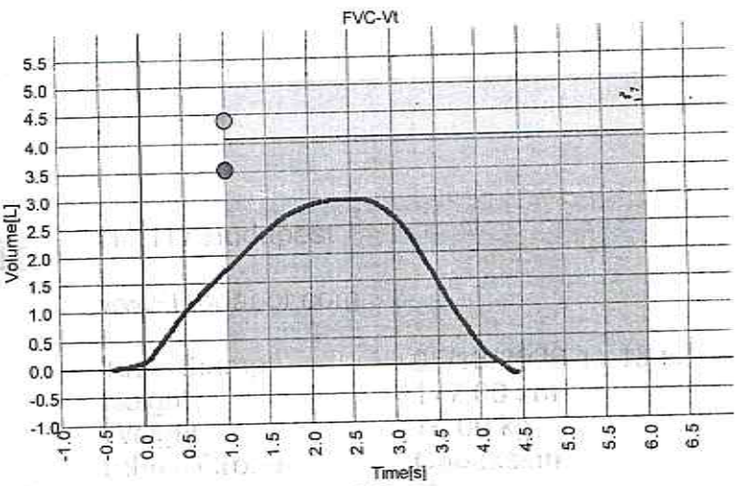
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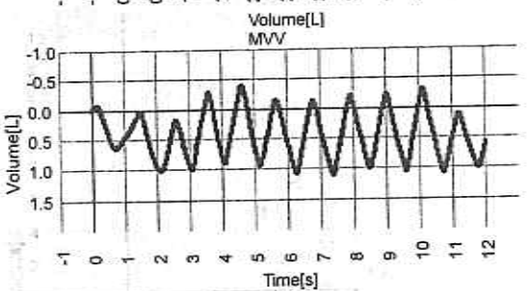
www.thorlabor.com

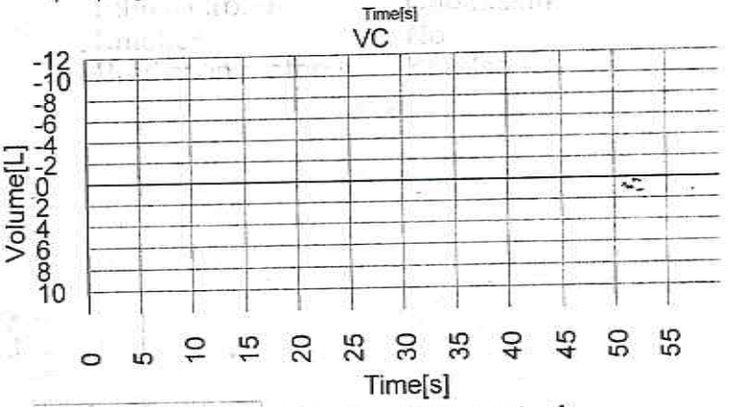
First name	kareem
Last name	ali
Birth date	06/02/2005
Id	876543
Age	20
Sex	Male

Time Stamp	05/13/2026 11:13:52
Height	177.00 cm
Weight	74.00 kg
Ethnic Group	Caucasian
Smoker	No
Prediction method	Knudson









	Unit	Predicted	%PRED	BEST
		-	-	☐
ATS/ERS		-	-	[illegible]
FVC	L	5.11	58%	2.98
FEV1	L	4.40	39%	1.73
FEV1%FVC	%	86.21	67%	58.05
PEF	L/s	9.23	23%	2.12
MMEF2575	L/s	4.91	31%	1.52
FEV05%FVC	%	-	-	32.89
FEV075	L	-	-	1.38
FEV075%FVC	%	-	-	46.31
FEV1%FEV6	%	-	-	58.05
FEV1%FIVC	%	86.21	63%	54.40
FEV3	L	-	-	2.98
FEV3%FVC	%	-	-	100.00
FEV6	L	-	-	2.98
Zero Time	s	-	-	0.36
ELA	years	20.00	-	-
EOTV05	L	-	-	0.36
EOTV1	L	-	-	1.04
EV	L	-	-	0.10
FEF25	L/s	8.42	23%	1.93
FEF50	L/s	5.64	24%	1.37

	Unit	Predicted	%PRED	Trial 1
MVV	L	131.50	45%	59.64

	Unit	Predicted
VC	L	5.11
IVC	L	5.11
IC	L	3.27
ERV	L	1.84
IRV	L	-
TV	L	-
TI	s	-
TE	s	-
VE	L/min	-
RR	breath	-
TV/TI	L/s	-
TE/TI	-	-

FVC Quality control

Automatic interpretation
Mild Obstruction

VC Maneuver
n/a

Doctor's Note

FVC:
MVV:

دروان عبد المنعم خضر
طبيب مقيم - قسم الصدر

Device Id	Firmware version	Calibration Verified at	Software version	Current version
ST-17110045	2.42	_____	ThorSoft 4.1.7.0	ThorSoft 4.1.7.0

[[DOCTYPE:PET_SCAN]]
PET Scan

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:	
Patient Name: Kareem MOHAMED ALI	*Date of birth:* 02/06/2005
Arabic Name: كريم محمد على	*Patient Gender:* M
Patient RIS ID: 367154 *Patient HIS ID:* 10110225003669	*Exam Date:* 12/02/2026
Written by: Dr. Hoda Fathy	**Approved by:** Dr. Hoda Fathy

Whole body PET/CT EXAMINATION **Date: (12-2-2026).**

Clinical History:

- 21 years old male patient diagnosed with managed DLBCL.
- **PET/CT requested for assessment.**

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis.

Report:

Compared to previous PET-CT dated: 1-9-2025.

Head and Neck:

* **Status post left parieto-temporal craniotomy showing progression of the non FDG avid left cerebral hypodensity, currently measuring~10.4 x 6.2 cm (Vs 6.2 cm); likely representing encephalomalacia with extra-axial bulge; for better morphologic characterization by MRI.**
* Normal appearance of the nasopharynx, oropharynx, salivary glands and larynx with no FDG avid focal lesions.
* Normal size, enhancement & symmetrical appearance of thyroid gland with no FDG avid lesions detected.

Chest:

* Few FDG avid right axillary lymph nodes, measuring~9 mm with SUVmax~25; representing benign changes due to extravasation.
* No FDG avid lung nodules or masses detected.
* No metabolically active pericardial or pleural lesions detected.

[[DOCTYPE:PET_SCAN]]
PET Scan

Abdomen and Pelvis:

* Physiologic diffuse FDG uptake is seen over average sized liver with no FDG avid focal lesions detected.
* Normal appearance of the spleen, kidneys, pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.

Musculoskeletal:

* Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

Conclusion:

PET-CT for a known patient with managed DLBCL shows:

- **Morphologic progression of the non FDG avid left cerebral hypodensity for better morphologic characterization by MRI.**
- **No FDG avid active nodal lymphomatous infiltrates detected.**

Much Obliged;
Dr. Aya Ashraf, MD
Nuclear medicine

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

المعهد القومي للأورام
جامعة القاهرة

Patient Name:	كريم محمد علي محمد		
MRN:	10110225003669	Location:	عيادة العلاج الاشعاعي
D.O.B:	02/06/2005	Sex:	Male
NC No.:	30506021702078		

Surgical Pathology-Diagnosis

Slide/Block revision Pathology

Specimen Id	2502241727	**Collection Date**	24/02/2025 06:11 PM
Accession No	P-25-2384	**Received Date**	24/02/2025 06:12 PM
Sample Type	TISSUE	**Doctor**	العلاج الاشعاعي

CLINICAL HISTORY

Block review of left parieto occipital SOL excisional biopsy

GROSS DESCRIPTION

The referred paraffin block labelled by patients name only.

MICROSCOPIC DESCRIPTION

There is diffuse effacement by infiltrate formed of noncohesive large atypical round cells having medium to large nuclei and multiple nucleoli with abundant cytoplasm. Anaplasia is moderate & Mitosis is active. Marker study was done with tumor cells are positive for LCA, CD20, BCL6 while negative for CMYC, BCL2, SALL4, GFAP, SYNAPTOPHYSIN, NKX2, BCOR. INI-1 IS RETAINED. KI67 is positive in 20 % of tumor cells. CD3 stains reactive background T cells. CD3 stained scattered few cells

DIAGNOSIS

Block review of left parieto occipital SOL excisional biopsy: Diffuse large B cell Non Hodgkin lymphoma

Category

Malignant primary site (single tumor)

Registrar:

Signed by:
Dr. Hazem Abdelazim, MD (Lecturer)

Consultant Pathologist:
Riham Gabr

Endorsed by Riham Gabr

[[DOCTYPE:LAB_TEST]]
Lab Test

الدكتـــورة
داليا حمدى توفيق طايع
بكالوريوس في الطب والجراحة
دكتوراه في تخصص الباثولوجيا الإكلينيكية والكيميائية
كلية الطب

المركز الطبى الخيرى
معمل رياض الصالحين
للتحاليل الطبية وأبحاث الدم والهرمونات

Visit No. : 156629
Patient ID : 7938
Male 21 Y

Request Date : 04/5/2026 05:23:52 PM
Patient Name : كريم محمد علي شحاته
Referred By : ----
Print Date : 04/5/2026 09:46:49 PM

COMPLETE BLOOD PICTURE

		Result	Unit	Reference Range
RED CELLS & HAEMOGLOBIN				
Hemoglobin	:	14.3	gm/dL	(13 - 16.5)
RBC-Erythrocytes	:	5.00	10^6/cmm	(4.5 - 5.5)
HCT-Hematocrit	:	40.2	%	(40 - 50)
MCV	:	80.6	fl	(80 - 100)
MCH	:	28.7	pg	(27 - 32)
MCHC	:	35.6	g/dL	(31 - 34)
RDW-CV	:	14.4	%	(11.6 - 14)
RDW-SD	:	46	fl	(Nil)
Platelet	:	262.0	10^3/cmm	(150 - 400)
PCT-Plateletcrit	:	0.18	%	(2 - 8)
MPV	:	7.00	fl	(1.7 - 7.5)
PDW-CV	:	16	%	(40 - 75)
WBC-Leucocytes	:	7.1	10^3uL	(4 - 11)
		Relative Count %		*Absolute Count X 10^3/ui*
Neutrophils	:	56 (45 -70)		3.98 (2 - 7)
Lymphocytes	:	33 (20 - 40)		2.34 (1.0 - 3.0)
Monocytes	:	7 (1 - 10)		0.5 (0.2 - 1.0)
Eosinophils	:	4 (1 - 6)		0.28 (0.02 - 0.5)
Basophils	:	0 (0 - 1)		0.00 (0 - 0.1)

Signature
elsayedomar

[[DOCTYPE:LAB_TEST]]
Lab Test

الدكتـــورة
داليا حمدى توفيق طايع
بكالوريوس في الطب والجراحة
دكتوراه في تخصص الباثولوجيا الإكلينيكية والكيميائية
كلية الطب جامعة المنوفية





المركز الطبى الخيرى
معمل رياض الصالحين
للتحاليل الطبية وأبحاث الدم والهرمونات

Visit No. : 156629
Patient ID : 7938
Male 21 Y

Request Date : 04/5/2026 05:23:52 PM
Patient Name : كريم محمد علي شحاته
Referred By : ----
Print Date : 04/5/2026 09:46:49 PM

CHEMISTRY REPORT

Test Name		Result	Unit	Reference Range
Kidney Function				
Serum Creatinine	:	1.13	mg/dL	(up to 1.3)
Liver Function				
SGOT (AST)	:	17	U/L	(Up to 42)
SGPT (ALT)	:	12	U/L	(Up to 42)
Total Bilirubin	:	0.56	mg/dL	(Up to 1.2)
Direct Bilirubin	:	0.15	mg/dL	(0 - 0.2)

Signature
elsayedomar

El-Batanon Medical Center

[[DOCTYPE:MRI]]
M.R.I

المعهد القومي للأورام
جامعة القاهرة

NCI

Patient Name:	Kareem MOHAMED ALI MOHAMED	Gender:	Male
MRN:	10110225003669	Ordered By:	
DOB:	02, 06, 2005 Age: 20 Y 6 M 25 D	Order ID:	2300060

MRI Brain with Contrast

Procedure Date: 23/08/2025 11:44:00 AM

Indications:

CONTRAST ENHANCED MRI EXAMINATION OF THE BRAIN

Findings:

- Status post-operative evidenced by left parietal craniotomy, showing.
- Overlying scalp swelling
- Underlying patchy area of encephalomalcia surrounded by gliosis; displaying low T1 and high T2 signal showing no diffusion restriction or contrast enhancement.
- No focal intra-axial enhancing lesions could be detected

- A large left perieto-occipital cystic lesion is seen measures 7x4.5cm with mass effect compression of left lateral ventricle and left uncal compression
- Right parietal shunt with asymmetrical dilation of lateral ventricles

الممسوحة ضوئيا بـ CamScanner

المعهد القومي للأورام

Patient Name: Kareem MOHAMED ALI MOHAMED		Gender: Male	
MRN:	10110225003689	Ordered By:	
DOB:	02, 06, 2005 Age: 20 Y 6 M 25 D	Order ID:	1523534

CT Neck Chest Abdomen Pelvis with Contrast

Procedure Date: 17/03/2025 12:12:00 PM

Indications:

CECT EXAMINATION OF THE NECK, CHEST, ABDOMEN AND PELVIS

A reported case of BLBCL at initial staging.

FINDINGS:

I.NECK

- Status post operative with evidence of left radical parotidectomy.
- Left frontal marginally enhancing SOL as seen surrounded by vasogenic edema compressing the left lateral ventricle, measuring about 32mm in maximum diameter. The lesion is seen interconnected with defined enhancing subcortical and periventricular white matter surrounded by moderate left fronto-parieto-temporal edema.
- Large temporal lobe cystic lesion- entrapped ventricle is also noted.
- Mild midline shift to the right side.
- Right ventriculo-peritoneal shunt tube is also noted.
- No sizable pathologically enlarged cervical lymph nodes.
- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Normal appearance of submandibular and parotid salivary glands as well as the thyroid gland.
- No neck soft tissue mass lesions.
- Normal appearance of the neck musculature & vessels.

II.CHEST

- No sizable pulmonary solid or mass-like areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.

III.ABDOMEN AND PELVIS

- The liver is of average size with homogenous parenchymal attenuation pattern. No focal lesions or contour irregularities. No intra-hepatic or extra-hepatic biliary ductal dilatation.
- Patent homogenously enhancing portal vein, hepatic artery branches as well as main tributaries.
- Normal size T- postcholecystectomy status. The upper pole has no focal lesions seen.
- Normal size, parenchymal attenuation & enhancement of both kidneys with no stones or back pressure changes.
- Normal appearance of the pancreas, spleen, both adrenal glands.

الممسوحة ضوئيا بـ CamScanner
CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name:	Kareem MOHAMED ALI MOHAMED	Gender:	Male
MRN:	10110225003669	Ordered By:	
DOB:	02, 06, 2005 Age: 20 Y 6 M 25 D	Order ID:	2455646

MRI Brain

Procedure Date: 01/12/2025 10:23:00 AM

Indications:

CONTRAST ENHANCED MRI EXAMINATION OF THE BRAIN

Findings:

compared to 23/8/2025

- Status post-operative showing:

grossly stationary purely cystic lesion with CSF signal and no enhancing nodules , seen measuring 4.6 x 5.2 x8 cm compared to 4.2 x4.5 x 7.5 cm previously,with still seen mass effect and surrounding encephalomalacia

- Right shunt tube with mildly dilated right lateral ventricle
- the left lateral ventricle is compressed by the cystic lesion

dr Manal refaat

CT Scan
[[DOCTYPE:CT_SCAN]]